



DOUBLE BAY
DAY HOSPITAL

Level 2 Commercial
451 New South Head Road
Double Bay 2028

DR Andrew FOWLER

MBBS (Hons), FRACP, MD

COLONOSCOPY REPORT

Date of procedure : 9 January 2025

Referring doctor: Dr Naomi Heath

Patient Name: Ms Shelley Lucy Manea

Anaesthetist: Dr Timothy Nelly

Patient DOB: 09/03/1950

Medication: Monitored Anaesthesia Care.

Indication	Positive Faecal Occult Blood Test under the National Bowel Cancer Screening Programme for investigation.
Preparation	The bowel was prepared with Picosalax. The quality of the bowel preparation was good.
MBS Item No	Positive FOBT - 32222 ; Polypectomy removal of 5 or more polyps - 32229
Extent of Exam	The instrument was inserted to the terminal ileum. Excellent views of the entire colon and terminal ileum were obtained. The procedure was completed without difficulty.
Findings	Small external haemorrhoids were identified. These were small and were not bleeding. Several diverticula were identified in the sigmoid colon. These were small and large in size and were not associated with muscular hypertrophy and luminal narrowing. There was no evidence of mucosal inflammation to indicate acute diverticulitis. Three polyps were identified in the right colon. The largest of these was approximately 3-6 mm in diameter and was sessile. Appearances were consistent with adenomatous polyps. These were completely removed using a cold snare and were all retrieved and sent for histology. Two large pediculated polyps and one small sessile polyp were identified in the sigmoid colon (approximately 15-20 mm in diameter). Appearances are consistent with adenomatous polyps. They were excised using a hot snare polypectomy technique and retrieved for histology. Prior to excision, the polyps were lifted using gelofusine, methylene blue and 1:10,000 adrenaline.
Specimens	Polyp(s): Three sigmoid colon polyps. Two ascending colon polyps. Caecal polyp x1

Diagnosis

- 1. Six colonic polyps were excised.**
- 2. Mild-moderate diverticular disease.**
- 3. Small haemorrhoids.**

Recommendation Mrs Manea will require a repeat colonoscopy in 3-years. I have placed her in our database and will send a reminder at the appropriate stage.

Await histological examination of the biopsy specimens taken.

Please arrange a follow-up review appointment with Dr George Marinos in 2-3 weeks (either a face-to-face or video telehealth) so as to discuss the findings of this examination, the results of the biopsies taken and treatment required, as well as to review the patient's clinical condition and to outline the need for any future examination or other investigations.

Dr Andrew Fowler

Patient: Ms Shelley Lucy Manea DOB: 09/03/1950 MRN : 0000



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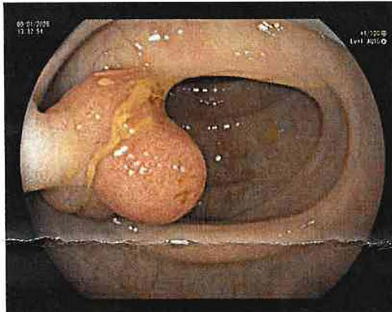
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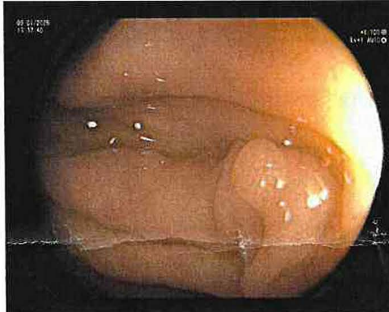
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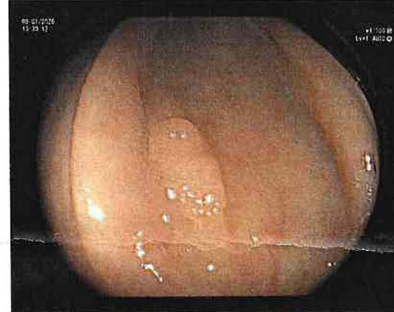
Referring doctor: Dr Naomi Heath



Sigmoid colon - polyp



Sigmoid colon



Caecal polyp



Caecal polyp



Caecal polyp



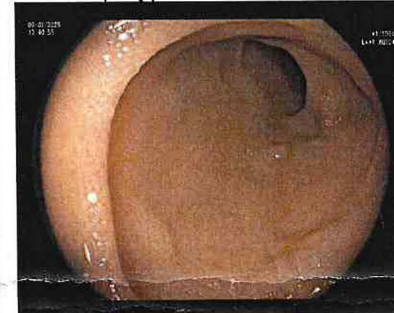
Caecal polyp



Caecal polyp



Terminal ileum



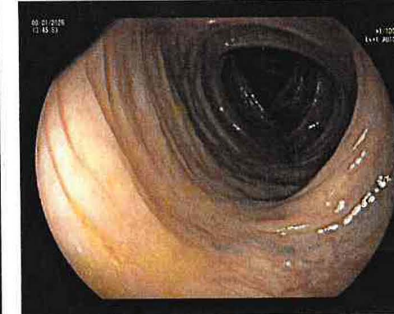
Terminal ileum



Ascending colon



Hepatic flexure



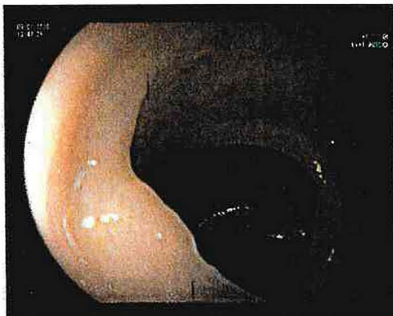
Transverse colon

Patient: **Ms Shelley Lucy Manea** DOB: 09/03/1950 MRN : 0000

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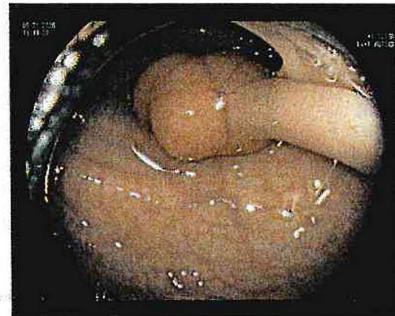
Referring doctor: Dr Naomi Heath



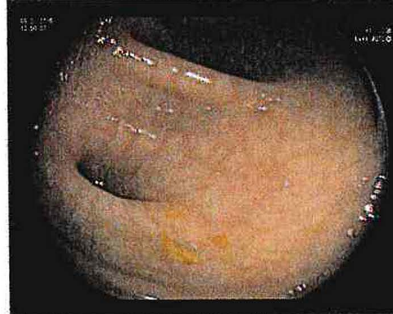
Sigmoid colon - polyp



Sigmoid colon - polyp



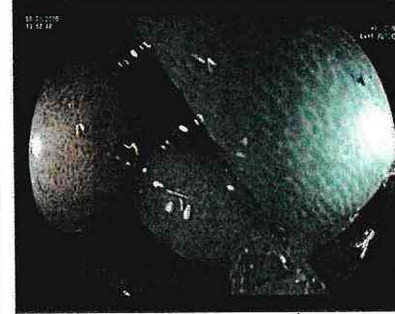
Sigmoid colon - polyp



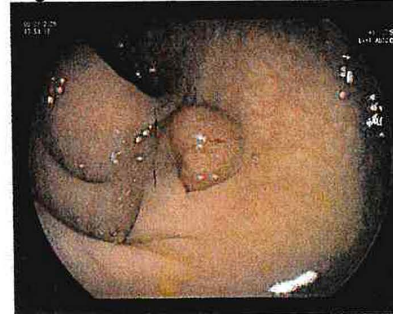
Sigmoid colon - diverticulae



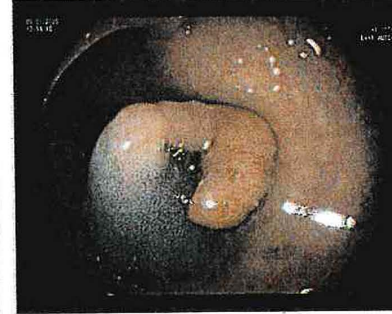
Sigmoid colon - diverticulae



Sigmoid colon - polyp



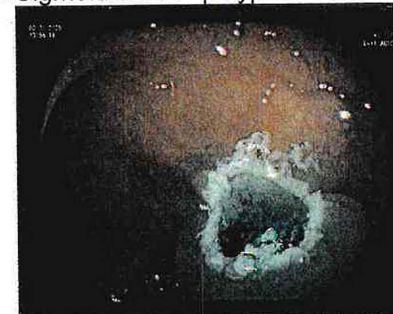
Sigmoid colon - polyp



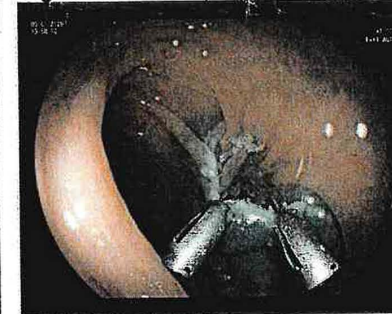
Sigmoid colon - polyp



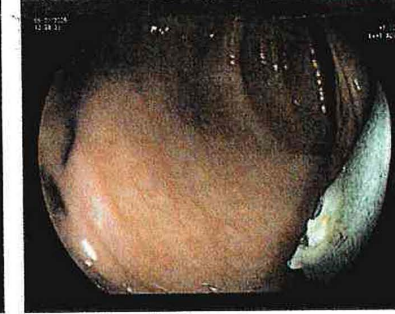
Sigmoid colon - polyp



Sigmoid colon - polyp



Sigmoid colon - polyp



Sigmoid colon - polyp



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DR Andrew FOWLER

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GASTROSCOPY REPORT

Date of procedure : 9 January 2025

Referring doctor: Dr Naomi Heath

Patient Name : Ms Shelley Lucy Manea

Anaesthetist: Dr Tim Nelly

Patient DOB : 09/03/1950

Medication: Monitored Anaesthesia Care

Indication Diagnostic procedure: Positive Faecal Occult Blood Test. Rule out active peptic ulceration. Rule out Coeliac Disease.

MBS Item No 30473 - GASTROSCOPY.

Exam Extent The endoscope was advanced to the second part of the duodenum. The oesophagus, stomach and upper duodenum were carefully inspected.

Findings

Oesophagus: The squamocolumnar junction was regular and seen at 40cm from the incisors. The oesophagus was normal. There was no evidence of erosive oesophagitis, Barrett's mucosa or a hiatus hernia.

Stomach: The stomach appeared endoscopically normal. There was no evidence of gastritis, peptic ulceration or any other mucosal abnormality.

Duodenum: The duodenal mucosa appeared endoscopically normal to the second part of duodenum.

Specimens Gastric biopsies from the antrum and body of stomach. Duodenal biopsies from 1st and 2nd parts of duodenum for histology.

Diagnosis **Normal Gastroscopy.**

Recommendation *Proceed to colonoscopic examination.*

Await histological examination of the gastric and duodenal biopsy specimens taken.

Please arrange a follow-up review appointment with Dr George Marinos (either face-to-face or video telehealth) in 2-3 weeks so as to discuss the findings of this examination, the results of the biopsies taken and treatment required, as well as to review the patient's clinical condition and to outline the need for any future examination or other investigations.

To arrange the review appointment with Dr Marinos,
-> book online: <http://bit.ly/dr-marinos> or call 02 9398 0200.

Dr Andrew FOWLER



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DR GEORGE MARINOS

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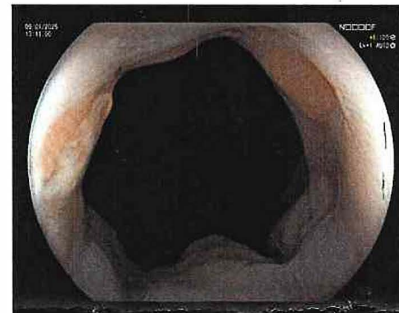
Referring doctor: Dr Naomi Jacobs



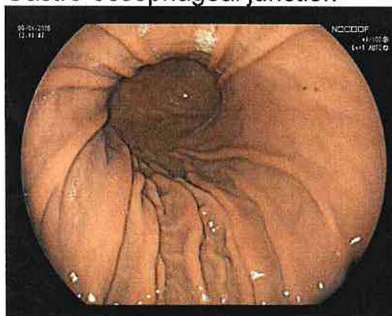
Gastro-oesophageal junction



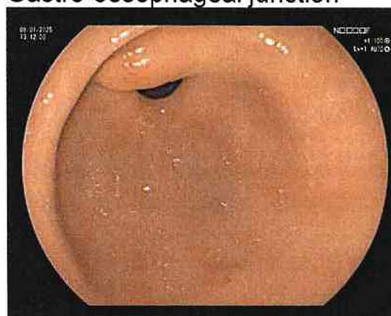
Gastro-oesophageal junction



Gastro-oesophageal junction



Gastric body



Gastric antrum



Gastric antrum



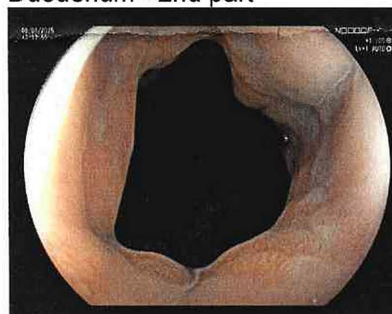
Duodenum - 2nd part



Gastric body



Gastro-oesophageal junction



Gastro-oesophageal junction